Pembrolizumab + pemetrexed + carboplatin

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: carboplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / brain

Pembrolizumab

Unusual tiredness, dizziness, headache, thirst or passing more urine.

2 Lungs

Pembrolizumab

New cough or breathlessness: report promptly. Infection, cancer or a clot can cause similar symptoms.

3 Heart

Pembrolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

4 Liver

Pembrolizumab

Yellow eyes or skin, dark urine or changes in blood tests.

5 Kidneys

Pemetrexed / pembrolizumab

Kidney function is monitored. Report less urine or new swelling.

6 Bowel

Pembrolizumab / chemotherapy

New diarrhoea, persistent vomiting or severe tummy pain: contact your team.

7 Skin / nerves

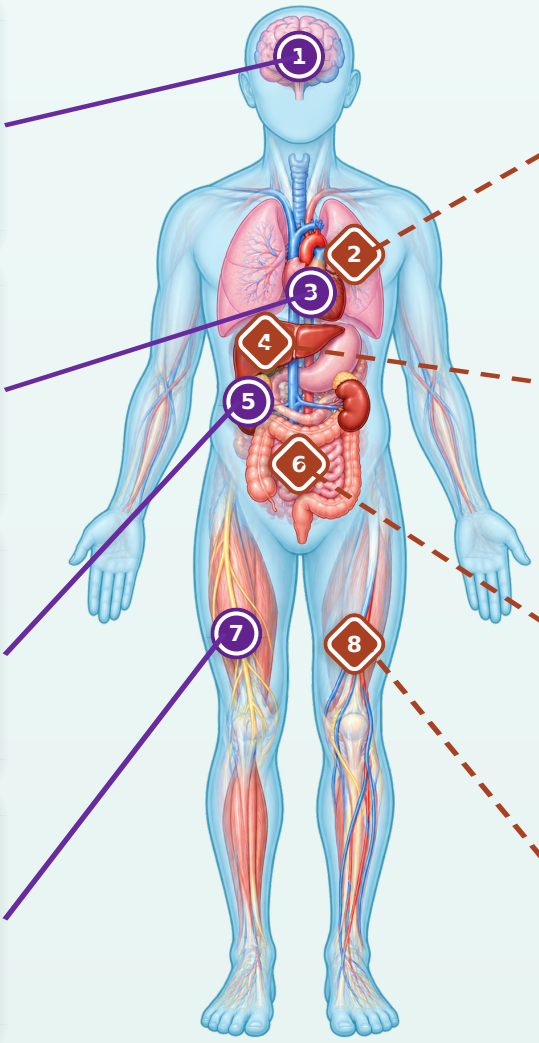
Pembrolizumab / chemotherapy

Spreading rash, blisters, weakness or new numbness should be reported.

8 Blood / infection

Pemetrexed / carboplatin

Fever, shivers or suddenly feeling unwell: contact oncology immediately.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for fever or feeling acutely unwell, new breathlessness or cough, severe diarrhoea, dehydration, chest pain, confusion, severe rash or other new symptoms. Use the emergency contact instructions given by your team.

Your treatment, at home.

Important medicine-specific effects

Pembrolizumab

- Immune inflammation of lungs, bowel, liver, kidneys, glands, skin and other organs
- Serious infusion reactions
- Effects can develop after treatment ends

Pemetrexed

- Low blood counts and infection
- Fatigue, nausea, mouth soreness and rash
- Kidney toxicity; folate and B12 supplementation are required

Carboplatin

- Low blood counts, infection or bleeding
- Nausea and vomiting
- Allergic reactions
- Kidney, hearing and nerve effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Induction: carboplatin-containing cycles 1–4

High emetic risk is driven by carboplatin AUC5. Dedicated NCIS carboplatin row has no routine olanzapine. Do not combine rash and antiemetic dexamethasone courses without reconciliation. Carry immunotherapy alert card; do not wait until next cycle for possible immune toxicity.

Maintenance: pembrolizumab + pemetrexed

Folate, B12 and rash prophylaxis continue because pemetrexed continues. Carboplatin ends after induction; review supportive prescriptions at the phase change. Continue renal, glucose, thyroid and immune-toxicity monitoring.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00568/

Sources and review status are available online. Follow your own prescription and hospital instructions.